Jess South
2026

In Search of Better Harm Reduction Data

Jess South, 2026

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Why harm reduction?

Harm reduction saves lives. It results in…

- better medical outcomes
- fewer drug-related deaths
- less long-term drug dependence

The data is clear. But one thing we're missing is a long-term modeled solution: The ability to determine how added and improved harm reduction solutions will directly impact a jurisdiction's outcomes.

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Available Target Data

Drug Death Rate

Drug-related deaths per 100,000 persons in a given area, including causes such as poisoning and long-term health effects.

Drug Dependency Rate

The self-reported percent of population aged 12 or older reporting illicit drug dependence or abuse in the prior year.

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Available Predictor Data

Syringe Exchange Program (SSP) Legality

Whether syringe exchange programs are legal in the locale (state).

Number of SSPs

The total number of SSPs in a given area.

Medicaid Coverage of Methadone

Whether Medicaid in the locale (state) will cover methadone treatment.

Number of FQHCs

Number of Federally Qualified Health Centers in a locale, which provide cost-adjusted substance abuse services.

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The available data is descriptive, not predictive.

The data available through sources such as CDC WONDER, amFAR, etc. is not sufficient for predictive modeling.

- To be predictive, we need **extensive time-series data** which can help determine the effect of harm reduction programs **over time in a given location.**
- Without time-series data, we can only see that the **correlation of harm reduction services and the number of recipients is high.**

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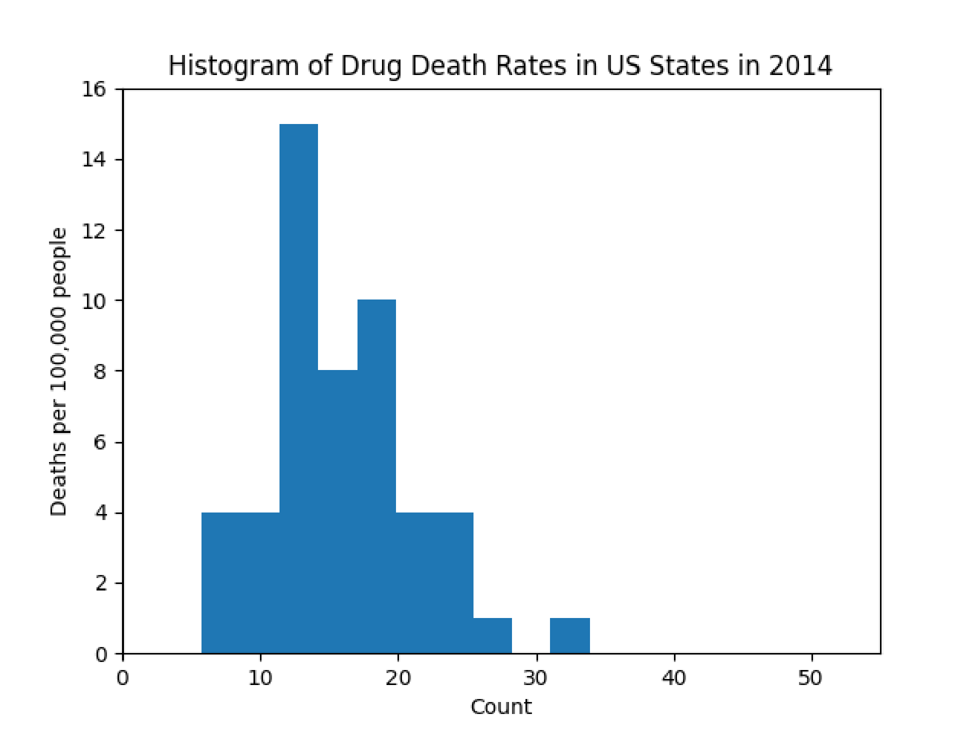
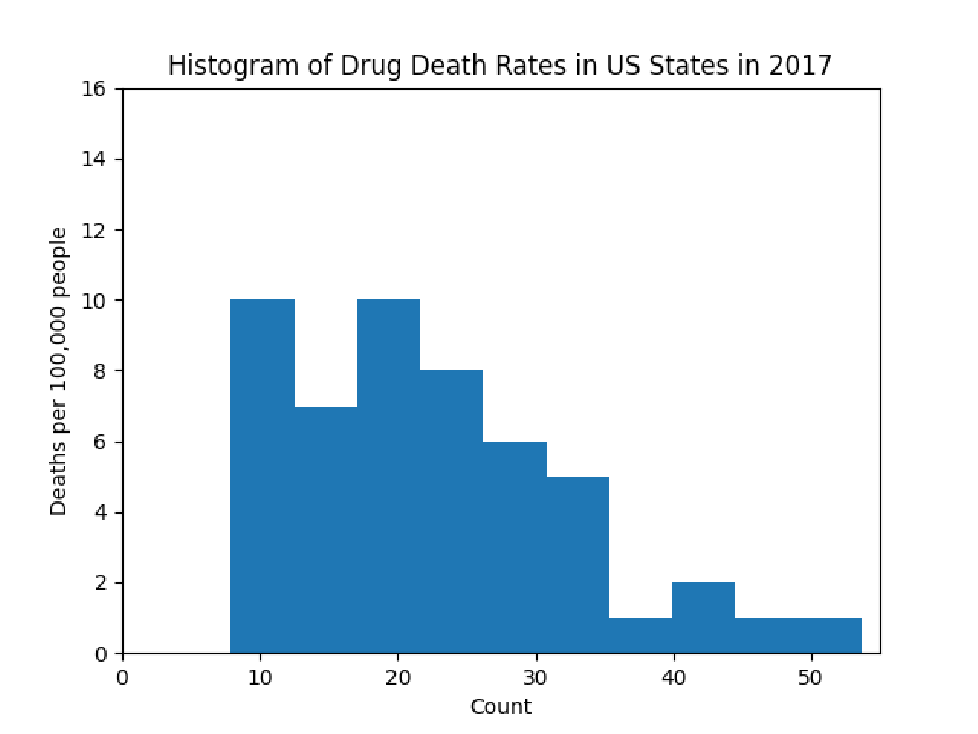


Death rates are meaningless without causal links.

We can see that drug-related deaths are becoming more of a problem in certain areas of the country, but it is difficult to contextualize this with harm reduction efforts.



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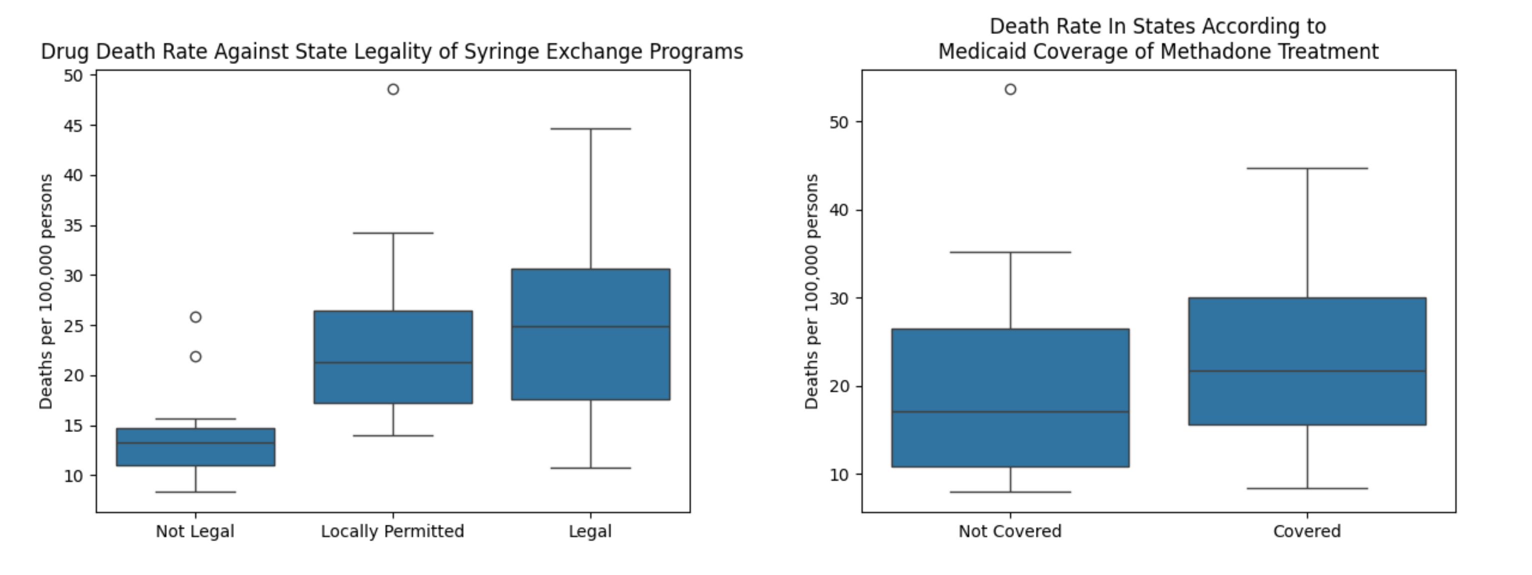
Targets and harm reduction services are correlated because of drug use density.



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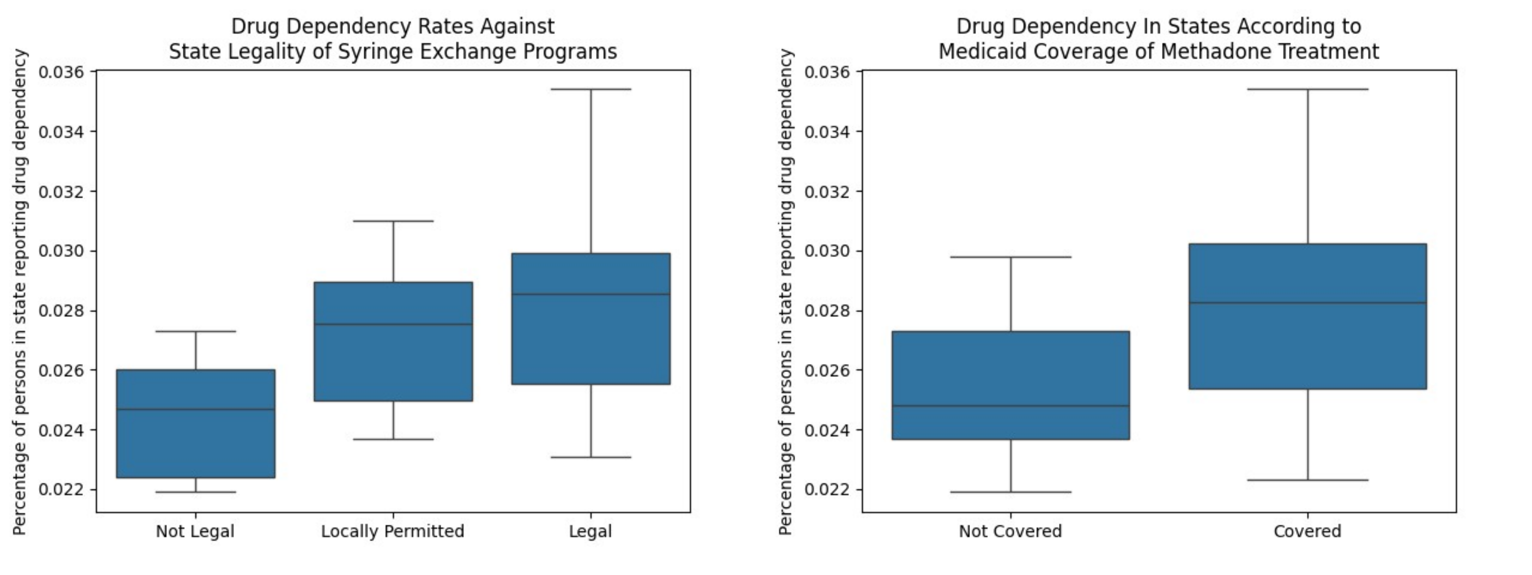

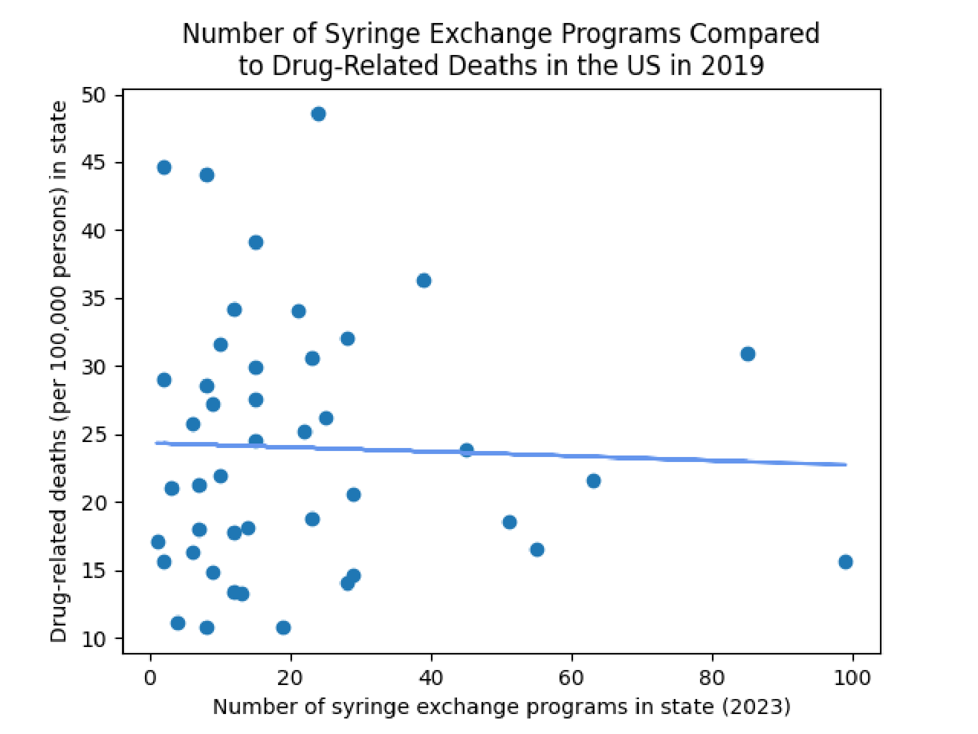

Without time-series local data, we can still see promises.

Even without accounting for changes in health outcomes over time, we can already see that harm reduction efforts such as SSPs are addressing health outcomes. However, without the time-series picture, these outcomes aren't significant enough to be predictive.

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The single biggest missing factor is local change over time.

What kinds of data would we need to collect to develop a predictive model?

- Time-series data would be able to ascertain the impact of harm reduction programs one, five, ten, etc. years after implementation.
- More robust legal data on the local level can take into account local implementations.
- Improved drug dependency data can help develop another significant training target.

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Implementing a Long Term Data Collection Initiative

What can we do with this information?

Step 1

Identify three to ten high-risk areas for drug-related adverse health outcomes where harm reduction programs are not yet implemented (such as counties which are legal but underserved).

Step 2

Implement harm reduction services such as an SSP or safe injection sites as a first pass. Leverage this service to collect data on drug use, dependency, health outcomes, and service use.

Step 3

Track outcomes over the course of one, five, and ten years.

Leverage this information to identify next-step predictor data to be studied.

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questions

thank you!

Let’s follow up! Email me at rabbitsouth@proton.me
for more project opportunities.